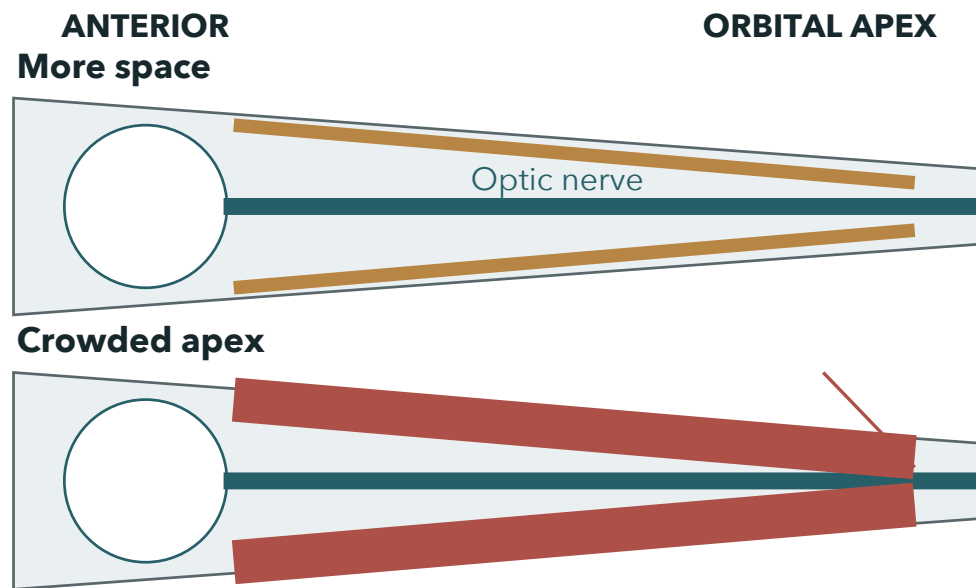


Thyroid eye disease: threat and rehabilitation

CLINICAL TYPE B | Focused decisions for M3-M5

Sight threat is a separate category

Crowding at the orbital apex



Enlarged tissues can compress the optic nerve

Original cross-sectional schematic, not to scale. Optic neuropathy may occur without striking proptosis.

Separate discomfort from nerve injury

Finding	Clinical inference
Surface symptoms	Check closure and corneal staining; lubrication and exposure protection help.
New acuity/colour/field deficit or RAPD	Suspect dysthyroid optic neuropathy; urgent specialist assessment and imaging.
Confirmed optic neuropathy	Immediate specialist IV glucocorticoids; early decompression if response is inadequate. Corneal breakdown also needs urgent treatment.

References EUGOGO: Graves orbitopathy guideline | ATA/ETA: Thyroid eye disease consensus

Rehabilitation follows disease stability

Conventional sequence when all stages are needed

- 1** Inactive disease with stable thyroid status and measurements: decide which residual problems warrant surgery.
- 2** Orbital decompression addresses proptosis/crowding and may change alignment.
- 3** Correct stable strabismus next; address eyelid position last because earlier operations can affect it.

While waiting for stable alignment

Finding	Clinical inference
Selected smaller deviations	Prisms may improve single vision in useful gaze positions.
Restriction	A fibrotic muscle can limit movement despite normal nerve function.
Elective surgery	The sequence is individualised; not everyone needs every operation.

Continue systemic care

Smoking cessation and stable thyroid control remain useful. Normal thyroid blood tests do not by themselves exclude thyroid eye disease.